

Code Blue — Adult Cardiac Arrest Response Protocol

Sunrise Medical Center

Document Title: Code Blue — Adult Cardiac Arrest Response Protocol

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Policy Owner: Director of Critical Care, Dr. Marcus Whitfield

Distribution: All Sunrise Medical Center Staff

1. Purpose and Overview

This protocol defines the standardized response to in-hospital adult cardiac arrest at Sunrise Medical Center, ensuring every unit follows an identical, rehearsed sequence of actions regardless of where the arrest occurs. Consistency across units reduces the cognitive load on responders during a high-stress event and improves return-of-spontaneous-circulation (ROSC) rates.

This protocol is based on the current American Heart Association Advanced Cardiovascular Life Support (ACLS) guidelines and is reviewed annually by the Resuscitation Committee.

2. Activation

Any staff member who discovers an unresponsive patient without a pulse or normal breathing must immediately call a Code Blue by pressing the nearest code button or dialing extension 5555 and stating the exact unit and room number twice.

- 1 Confirm unresponsiveness and absence of normal breathing within 10 seconds.
- 2 Call Code Blue via extension 5555 or the nearest wall-mounted code button.
- 3 Begin high-quality chest compressions immediately — do not wait for the code team to arrive.
- 4 Send a second staff member to retrieve the nearest crash cart and AED/defibrillator.
- 5 Designate a team leader once the code team arrives; the first responder briefs the leader on down time and interventions already performed.

3. Code Team Composition

The Sunrise Medical Center Code Blue team consists of the on-call Hospitalist or Intensivist (team leader), an ICU or ED nurse (medication nurse), a Respiratory Therapist (airway management), a second nurse (compressions/documentation), Pharmacy (on radio standby), and Security (crowd and family management). Response time target is under 3 minutes from activation to bedside arrival for all core team members.

4. Resuscitation Sequence

The team follows the standard ACLS algorithm: continuous high-quality compressions at a rate of 100–120 per minute with a depth of at least 2 inches, minimal interruptions, early defibrillation for shockable rhythms,

and airway management concurrent with compressions rather than sequentially.

- Compressor rotation occurs every 2 minutes (or sooner if fatigue is evident) to maintain compression quality.
- Rhythm checks occur every 2 minutes with compressions resuming immediately after the check unless a shock is indicated.
- Epinephrine 1 mg IV/IO is administered every 3–5 minutes per algorithm; amiodarone or lidocaine is considered for refractory ventricular fibrillation or pulseless ventricular tachycardia.
- Reversible causes (the H's and T's) must be verbally reviewed by the team leader at least once during any code lasting longer than 10 minutes.

5. Post-Resuscitation Care

Patients achieving ROSC are transferred to the Intensive Care Unit for targeted temperature management evaluation, continuous cardiac monitoring, and 12-lead ECG to assess for STEMI requiring emergent catheterization. The receiving ICU team must be briefed using the SBAR format before transport begins.

6. Family Notification and Presence

A designated staff member, typically Chaplaincy or Social Work when available, is assigned to remain with family members throughout the event. Family presence during resuscitation is permitted per hospital policy SMC-PT-007 provided a support staff member is present to explain ongoing interventions.

7. Documentation

The code recorder documents all interventions, medication times and doses, rhythm checks, and team members present on the standardized Code Blue Record. This form is scanned into the patient's chart within one hour of code conclusion and a copy is forwarded to the Resuscitation Committee for review.

8. Debrief

A structured hot debrief is conducted immediately following every code, led by the team leader, covering what went well, what could improve, and any equipment or systems issues. This is distinct from the formal case review conducted by the Resuscitation Committee within two weeks.

9. Equipment and Crash Cart Standards

Item	Check Frequency	Responsible Party
Defibrillator/AED function test	Daily	Unit Charge Nurse
Crash cart seal/lock verification	Daily	Unit Charge Nurse
Medication expiration audit	Monthly	Pharmacy
Airway equipment inventory	Monthly	Respiratory Therapy

10. Related Documents

SMC-CC-004 Post-Cardiac Arrest Targeted Temperature Management Policy; SMC-PT-007 Family Presence During Resuscitation Policy; SMC-PHARM-011 Code Cart Medication Standards; SMC-RT-002 Airway Management Equipment Manual.

Appendix A: Frequently Asked Questions

Q: Who can serve as team leader if the on-call Hospitalist is delayed? A: The most senior physician present, including an Emergency Physician or Anesthesiologist, assumes team leadership until the designated leader arrives, at which point a formal handoff of leadership occurs verbally.

Q: What if the defibrillator pads are already applied by EMS on arrival? A: Continue using the existing pads if functional and compatible with hospital equipment; do not delay rhythm analysis to swap pads unless they are damaged or improperly placed.

Q: How is a DNR/DNI status confirmed during an active code? A: Compressions continue until a valid, chart-confirmed DNR/DNI order is verified by the team leader; verbal family reports alone, without chart documentation, are not sufficient to withhold resuscitation.

Q: What is done with unused code cart medications afterward? A: Any opened vials are documented and wasted per pharmacy controlled-substance procedure; the cart is restocked and resealed by Pharmacy before returning to the unit.

Appendix B: Common Scenarios

Scenario — Witnessed arrest with shockable rhythm: Immediate defibrillation takes priority over drawing up medications; the first shock should occur as close to rhythm identification as possible, ideally within 2 minutes of collapse.

Scenario — Pregnant patient in cardiac arrest: Manual left uterine displacement is added to standard compressions, and Obstetrics is paged immediately given the possibility of emergent perimortem cesarean delivery if ROSC is not achieved within 4 minutes.

Scenario — Code called in a public area (cafeteria, lobby): Security is dispatched immediately to secure the area and guide the responding code team to the fastest route, since public-area codes often have longer team travel times.

Revision History

Version	Date	Description
1.0	Jun 1, 2021	Initial release
1.2	Aug 20, 2023	Updated compressor rotation interval
1.3	Jan 8, 2026	Added post-arrest STEMI catheterization pathway